



# A Hypnosis-Informed Framework for Post-Psychedelic Integration

Stability, autonomy and referral boundaries

**Angélica Angaramo Chumachenko**

Angelux Hypnose, France. Hypnotherapist, independent practitioner

Practice-based methodological perspective

FRANCOPHONE  
ÉCHANGES EN FRANÇAIS BIENVENUS

French-speaking presenter

CORE PROPOSITION

**Post-psychedelic integration cannot replace preparation and a safe framework. Responsible care requires verifiable competence, clear safety procedures and follow-up. When severe instability is present, stopping and urgent referral protect the individual.**

CONTEXT

Psychedelic experiences may profoundly alter perception, emotion, identity and meaning. The work that follows concerns how the person carries the experience into ordinary life.

WHY WORK IN A HYPNOTIC STATE?

Within the framework presented, change is understood as occurring at the same moment across three inseparable levels: emotion, brain and body. Working in a hypnotic state allows the person to access these three dimensions simultaneously, create new connections between them and build a new internal foundation. This new foundation then supports the integration of the psychedelic experience into everyday life.

PRACTICE FRAMEWORK

1

Initial assessment

Assess current stability, expectations, the relationship to the experience and my scope of practice.

2

Stabilisation

Support emotional and bodily regulation before deeper exploratory work.

3

New internal foundation

Develop new links between the experience, self-understanding and the surrounding world.

4

Integration and reassessment

Extend the new perspective into daily life. Continue only while a stable basis is present.

PRACTICE SITUATION A

BUILDING A NEW INTERNAL FOUNDATION

**An individual came to me after psychedelic experiences conducted under psychiatric supervision in Geneva.**

Initial challenge

She said she wanted to return to her psychiatrist, not to feel better, but to recover the altered state and the sense of bliss she missed in daily life. This motivation concerned her: she recognised the risk of becoming reliant on further appointments to access that state again.

How we worked

During our work in a hypnotic state, conducted without any substance effects, we did not try to recreate the psychedelic state. Together, we developed new links between the experience, her understanding of herself and her view of the surrounding world.

WHAT THE WORK HELPED TO CONSTRUCT

**This new internal foundation allowed the emerging perspective to extend into everyday life. She could begin to live from this new view without needing to return to the psychedelic state to access it.**

PRACTICE SITUATION B

AN ALERT ABOUT PREPARATION AND SUPPORT

**According to his account, the individual I saw had attended two sessions with someone presenting themselves as a shaman, who had given him psychedelic mushrooms. He reported receiving neither structured preparation nor post-experience follow-up. The title ‘shaman’ alone did not allow the facilitator’s training, competence or safety procedures to be verified.**

What I observed

I observed marked activation, hyperactivity, instability, disorientation and overwhelming internal bodily sensations. People around him reported a profound change from his usual behaviour. He struggled to understand why others did not share his perception, described himself as ‘Alpha’ and expected others to follow him.

PREVENTION AND MY PROFESSIONAL DECISION

**This situation highlights the need to choose a setting through verifiable competence, prior assessment, preparation, safety procedures and follow-up, rather than a fashionable title or a lower price. After two sessions, I stopped the work and recommended urgent psychiatric assessment because I could not establish a stable, safe basis within my scope.**

Practice-based observations from two anonymised situations.  
No claim of clinical efficacy or causality.

PRACTICE AND ETHICAL IMPLICATIONS

Preparation belongs to safety

Responsible practice begins before the experience, with careful assessment and preparation.

A title is not a safeguard

The word ‘shamanic’ alone does not establish competence, accountability, safety procedures or follow-up.

Severe instability changes the priority

When a stable basis cannot be established, urgent assessment and specialised care take priority over integration.

PROFESSIONAL CLARITY AND COURAGE

**Responsible practice requires the clarity to recognise when an intervention has reached its limits and the courage to stop. Ending the work at the right moment protects the individual and enables prompt referral to appropriate care.**

CONCLUSION

**It is time to discuss psychedelic substances openly, beyond taboos and promises of magical transformation. The framework of care must become as visible as the substances themselves. Popularity, ease of access or price alone cannot establish its quality. Clear information about possibilities, limitations and risks supports informed choices. The individual’s psychological and physical health must remain at the heart of those choices.**

BOOKS INFORMING THIS PERSPECTIVE

Duffour C. Kétamine. Conscience. Thérapie : Les psychédéliques à la lumière de mes patients. Éditions Rosso; 2025.  
Nora D. Voyage dans les médecines psychédéliques. Paris: Grasset; 2025.  
Pollan M. Voyage aux confins de l’esprit. Lausanne: Quanta; 2019.  
Wasson VP, Wasson RG. Mushrooms, Russia and History. Vols. I-II. New York: Pantheon Books; 1957.

METHOD NOTE

All integration sessions occurred sober. I did not provide, administer or facilitate psychedelic substances. All situations are anonymised.



SCAN TO DISCOVER  
MY WORK